



Hospital AI Scribe Executive Scorecard

Authorize the benefit you can defend.

A proposed local tool, not the study's evaluation instrument or a validated clinical standard. Separate documentation quality, workforce relief, capacity and realized cash.

ORGANIZATION / EVALUATION LEAD

SCRIBE / EHR / EVALUATED SYSTEM VERSION

SPECIALTIES / ENCOUNTER TYPES / LANGUAGES / SEAT COUNT

BASELINE AND EVALUATION DATES / EQUAL SIGNING FOLLOW-UP

CAPPED BUDGET / END DATE / CONTRACT EXIT OR RESIZING TERMS

Investment purpose (select all applicable)

- ☐ Documentation quality
- ☐ Workforce relief
- ☐ Added capacity
- ☐ Realized financial return

Counting rules: Fix eligibility independently of actual use. Deduplicate by encounter ID. Include failures and opt-outs; report late or unsigned notes separately. Use equal follow-up. Do not let pooled averages hide a failing workflow.

Response handling: Use aggregate data, not patient identifiers. Save a completed copy in a PDF viewer with form support. Calculations are manual; no automatic totals or universal pass score are supplied.



Adoption & Clinical Content

01 Usable adoption

unique eligible encounters with at least one scribe-assisted note signed within the fixed window / all unique eligible encounters; separate attempt, failure, and opt-out counts.

Owner: Ambulatory operations | **Data:** Encounter roster, scribe logs, EHR linkage

Decision use: Size seats to actual use; prevent duplicate-note inflation.

BASELINE / DENOMINATOR / PERIOD

EVALUATION / DENOMINATOR / PERIOD

MISSING OBSERVATIONS / SPECIALTY AND CLINICIAN VARIATION

PRE-AGREED LOCAL CRITERIA / EVIDENCE REFERENCE / NEXT OWNER

- ☐ Evidence pending
- ☐ Local criteria met
- ☐ Local criteria not met

02 Clinical content

notes with clinically consequential errors / audited notes, separately for drafts and signed notes; complete required elements / applicable required elements.

Owner: Clinical quality lead | **Data:** Authorized encounter evidence, chart, independent audit

Decision use: Detect introduced errors and failures that survive review.

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- ☐ Local criteria not met



Work & Realized Capacity

03 Net documentation work

documentation-active person-minutes of clinicians and documentation-support staff / observed completed encounters, versus comparable baseline; report each labor group and after-hours minutes separately.

Owner: Clinical informatics | **Data:** Time observation, schedules, EHR activity logs

Decision use: Test work reduction versus transfer, including failed-attempt fallback.

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☐ Evidence pending

☐ Local criteria met

☐ Local criteria not met

04 Realized capacity

incremental completed visits / comparable staffed sessions; adjust for case mix and scheduling changes.

Owner: COO | **Data:** Scheduling, staffing, encounter records

Decision use: Test conversion of time into throughput.

BASILINE / DENOMINATOR / PERIOD

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☐ Local criteria met

☐ Local criteria not met



Benefit & Incremental Cost

05 Realized financial benefit

verified expense reductions plus incremental collected contribution, in dollars per evaluation period.

Owner: CFO/controller | **Data:** Payroll, general ledger, collections, visit costs

Decision use: Separate observed cash benefit from forecasts.

BASELINE / DENOMINATOR / PERIOD

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- ☐ Local criteria met
- ☐ Local criteria not met

06 Incremental cost

total dollars and dollars / eligible encounter; separate one-time, recurring cash, and staff opportunity costs.

Owner: Finance with CIO | **Data:** Contract, invoices, implementation and support time

Decision use: Establish full cost and volume sensitivity.

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Control Exceptions & Counting Rules

07 Control exceptions

unreviewed released notes / released scribe-assisted notes; failed required audit checks / required checks performed; privacy incidents as distinct counts by severity, with an exposure denominator only when meaningful and defined.

Owner: CIO/CAIO with privacy lead | **Data:** Release logs, incident register, version history

Decision use: Escalate serious events regardless of aggregate rates.

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Work minutes, not elapsed logins

Count each person's active time once across setup, review, correction and failed-attempt fallback. Passive capture concurrent with care adds no work minutes. Overlapping review/correction counts once; sum simultaneous work by different people. Report active after-hours person-minutes per clinician-day using scheduled hours.

Keep cash and opportunity cost separate

Cost licenses, usage, interfaces, training, support, audits, security review and fallback. Include incremental paid correction labor once in cash costs. Include existing salaried correction effort once only in the opportunity-cost view. Measuring minutes and costing them are not double counting.

Reconcile the financial case

Cash case: verified expense reductions plus incremental collected contribution, less incremental cash costs. Deduct added visit-delivery costs within contribution and scribe costs separately, once. Separate uncollected forecasts. Value existing staff effort at agreed loaded rates without recounting labor already in cash costs. Show first-year and steady-state economics separately.



Expansion Decision

EXPAND

Only tested workflows meeting pre-agreed local quality and control criteria, with credible net-work improvement and an affordable, explicit benefit case. Workforce-relief authorization is distinct from financial-return authorization.

CONTINUE WITHIN THE CAP

When quality controls hold but adoption, time estimates or collections remain uncertain. Set a specific evidence gap and deadline; do not renew indefinitely.

PAUSE & ESCALATE

Affected workflows with a predefined serious-error or privacy trigger, mandatory-review failure, or a version change invalidating evaluation. Low aggregate rates do not override triggers. Resume after documented remediation and retesting.

- ☐ Expand
- ☐ Continue within cap
- ☐ Pause and escalate

BENEFIT AUTHORIZED / LOCAL EVIDENCE / CONDITIONS AND UNCERTAINTY

MISSING EVIDENCE / ACCOUNTABLE OWNER / NEXT DEADLINE / COLLECTIONS RECONCILIATION

AFFECTED WORKFLOW / ESCALATION / REMEDIATION AND RETESTING, IF APPLICABLE

LOCAL DECISION OWNER / MEETING DATE (NOT ALHQ ASSET APPROVAL)

No threshold, safety certification or automated ROI is implied. A short operational evaluation is not a patient-outcome trial. Apply locally approved criteria, not a universal similarity cutoff.